Patient Cost-Estimate Automation

A HIPAA-compliant workflow for turning a day's scheduled-patient roster into per-patient cost estimates using ModMed, Availity Essentials, and DoxGPT.

A step-by-step guide to pulling your day's Eligibility Report out of ModMed, verifying eligibility for every patient in a single batch through Availity (split by payer), and generating per-patient cost estimates with DoxGPT — all within a HIPAA-compliant chain of custody. The reference example uses ModMed and Availity; the architectural pattern adapts to other EMRs and clearinghouses via the companion AI Reference document.

What You Need

ModMed EMA with **Eligibility Report** accessible from Appt Flow (this is included in standard ModMed configurations — look for the *View Eligibility Report* link with a badge on the right side of the date header). An active Availity Essentials account with Eligibility & Benefits — Batch enabled (included with most Availity subscriptions). A Doximity account with DoxGPT access (free for verified U.S. clinicians). Google Chrome on a Mac or PC. About 20 minutes for the full end-to-end workflow.

> **HIPAA Note:** PHI in this workflow stays inside three vendors that each hold a Business Associate Agreement covering your use — ModMed, Availity, and Doximity. The browser script in Phase 1 runs locally and never transmits anything. The chain is unbroken end to end. **Personal Gmail, generic ChatGPT/Claude/Gemini accounts, and consumer texting apps are NOT in this chain.** Do not paste Availity output into any of them. Module 10 of the full course covers the email side of this.

Phase 1: Export the Eligibility Report from ModMed

Step 1: Open the Eligibility Report

Log in to ModMed EMA. Click **Appt Flow** in the top navigation. On the right side of the date header (where it says e.g. *Friday, May 29, 2026*), click **View Eligibility Report**. The page loads with all scheduled patients for the active filter, with insurance information already attached to each.

Step 2: Set the Filter and Pagination

Set the filter to the date range you want verified — most practices run this once a week for the upcoming 7-day window. Set pagination to show **ALL rows** (highest available value in the per-page dropdown). The script reads only what is visible — anything hidden behind pagination is skipped.

Step 3: Open the Chrome Developer Console

Press **Cmd+Option+J** (Mac) or **Ctrl+Shift+J** (Windows). Chrome will show a warning about pasting code into the console. Type **allow pasting** and press Enter.

The first time you run this script on a multi-payer day, Chrome will also prompt: *"This site is attempting to download multiple files."* Click **Allow**. Chrome remembers this for the rest of the session.

Step 4: Run the Extraction Script

Open the file **1_Extract_Eligibility_Report.js** from this bundle. Select all the text (Cmd+A / Ctrl+A), copy it, paste it into the Chrome console, and press Enter.

The script walks the Eligibility Report table, groups patients by payer, and writes **one CSV per unique payer** to your Downloads folder. Each CSV is in the exact format Availity's batch eligibility intake expects: `last_name, first_name, dob, member_id, payer, provider_npi`.

Filenames are payer-derived, e.g.:

- `availity_medicare_of_texas.csv`
- `availity_bcbs_ppo.csv`
- `availity_aetna_choice_pos.csv`

Watch the console output. You should see an extraction summary listing each payer and patient count per payer, followed by one *Downloaded: ...* line per file.

Phase 2: Run the Batches Through Availity

Step 5: Log in to Availity Essentials

Open availity.com and sign in. Navigate to **Patient Registration** → **Eligibility and Benefits**, then look for the **Batch** or **Bulk** submission option. Availity occasionally renames menu items, so the exact path may differ; what you are looking for is the option to upload a CSV of multiple patients rather than entering one patient at a time.

Step 6: Upload Each Payer CSV Separately

Upload your CSVs **one at a time**, one per payer. Availity's batch endpoint runs against one payer's roster per submission, so mixed-payer uploads will either be rejected or processed slowly. Splitting by payer up front matches Availity's expected workflow and dramatically speeds processing.

Availity will validate each file and flag any malformed rows — missing member IDs, unrecognized payer names, badly formatted dates of birth. Fix those rows directly in the CSV (or in ModMed) and re-upload only the failed subset.

Step 7: Submit and Wait

Submit each validated batch. Availity processes most batches in about three minutes; a 100-patient batch typically returns inside five. Per-payer response time varies, and an occasional payer is slow enough that some rows return as **Pending**. Pending rows resolve within a few hours — you can re-export the batch result later and re-process the subset that was pending.

Step 8: Download the Response

When each batch completes, download the response CSV. The response will include, for each patient: member status, plan name and type, copay tier, deductible amount and remaining balance, out-of-pocket maximum and remaining, in-network coinsurance percentage, and any prior-authorization flags on the plan. These response CSVs are the input for Phase 3.

A de-identified sample of what an Availity response looks like is included in this bundle as **Sample_Availity_Output.csv** — open it now to see the data shape.

Phase 3: Generate Per-Patient Cost Estimates with DoxGPT

Step 9: Open DoxGPT and Load the Prompt

Sign in to doximity.com and open **DoxGPT** (Doximity Ask). Start a new chat. Open the file **DoxGPT_Prompt_Template.md** from this bundle and paste the prompt block (from `You are` through `front-desk summary.`) into the DoxGPT chat box. Do not hit Ask yet.

> **Important:** Before you use this in production, edit the four bracketed sections in the prompt to match your practice — practice name, fee schedule, planned procedures bundle, and the mutually-exclusive procedures note. The customization checklist inside the template walks you through it. The fee schedule shown in the template is illustrative placeholder values, not real contracted rates — you must replace every value with your own contracted allowables.

Step 10: Paste the Patient Row

Copy a single row from your Availity output CSV — including the header row, so DoxGPT knows which column is which — and paste it directly below the prompt. Alternatively, click the paperclip icon and attach the full Availity output PDF and ask DoxGPT to process a specific patient by name.

Hit **Ask**.

Step 11: Review the Output

DoxGPT returns:

- A one-line patient summary

- A per-procedure breakdown (CPT, description, allowable, applied to deductible, coinsurance, copay, owed)
- The total to collect up front today, showing both scenarios where procedures are mutually exclusive and recommending the conservative (higher) one
- Flags (auth required, inactive coverage, pending) requiring staff review
- A one-line front-desk-ready summary you can paste directly into the appointment note or read aloud to the patient at check-in

Step 12: Repeat or Batch

For a single patient, that is the workflow. For a whole batch, you have two options:

- **Run patients one at a time** if you want to review each estimate before it goes into the chart. About 30 seconds per patient.
- **Attach the full Availity output and ask DoxGPT to process every row** if you trust the prompt's guardrails for your specialty. About 2-3 minutes for a 40-patient batch. Spot-check 3-5 rows for sanity before using the rest.

Troubleshooting

"Cannot find #reportTable" — You are not on the Eligibility Report page, or the table has not finished loading. Navigate to Appt Flow → View Eligibility Report and wait for the table to render before running the script.

"Patient column not found" — The table loaded, but its column headers do not match what the script expects ("Patient", "Payer", "Policy Number"). If ModMed has renamed columns in your configuration, adjust the header-matching lines near the top of the script.

Script returns 0 rows — Pagination is likely still set to a low number, or your filter is too narrow. Set pagination to "show all" and confirm the rows you want are visible, then re-run.

Chrome blocks the downloads — On the first multi-payer run, Chrome prompts to allow multiple downloads. If you missed the prompt, click the small icon in the address bar (a download or shield icon) and grant permission, then re-run the script.

Availity rejects an upload — Check the validation error. Usually a malformed DOB (Availity wants MM/DD/YYYY), a missing member ID, or a payer name Availity does not recognize. The payer names in your CSVs come straight from ModMed; Availity uses its own internal payer list, so occasionally a name mismatches. Fix in the CSV and re-upload.

Some Availity rows come back as Pending — Normal. The payer has not responded yet. Re-run the batch in a few hours and just process the rows that were pending.

DoxGPT output is missing fields — The Availity row you pasted is missing those fields. Pending rows have null deductible / coinsurance / OOP data. DoxGPT correctly refuses to fabricate a number and flags the patient for re-check. Do not "fill in" the missing data manually.

DoxGPT estimate looks wrong — First, verify the fee schedule in your prompt matches your actual contracts. Second, verify the planned procedures bundle reflects what you actually perform. Third, verify the mutually-exclusive procedures note is complete. The prompt is only as accurate as the four customization sections.

What This Replaces

Before automation, the same daily roster looks like this: a front-desk person opens Availity, looks up each patient one at a time, copies the deductible and copay information into a spreadsheet, then opens a calculator and works out an estimate using a separate fee schedule. About six to eight minutes per patient. For 40 patients a week, most of a workday.

With the automation: about 15 seconds of script time in Phase 1, three to five minutes of Availity processing per payer in Phase 2 (parallelizable — you can submit all payer batches at once), and 30 seconds per patient in Phase 3 (or a single batch run if you trust the guardrails). For 40 patients, the total clock time is under 30 minutes — most of which is Availity processing while you do something else.

The compliance posture is also stronger than the manual workflow. Manual lookup means a staff member is reading patient eligibility information on screen and re-typing it into a spreadsheet — a step where transcription errors quietly enter the chart. The automated workflow moves structured data between vendors that already have BAAs with you, with no manual re-typing in the middle.

Companion document: AI Reference — DoxGPT Cost Estimate Pipeline. Course module: Module 5 — Insurance Verification & Patient Cost Estimates.